

CTA CORONARY ARTERIES: 8/8/2025 3:22 PM

COMPARISON: Cardiac MRI 8/7/2025.

PROCEDURE COMMENTS:

Premedication: None.

Intravenous Beta-blocker: None.

Nitroglycerine: None.

Image Acquisition by Series:

1. Coronary Angiogram

Dose information: Based on a 32 cm phantom, the estimated radiation dose (CTDIvol [mGy]) for each series in this exam is 0.01,0.32,0.32,4.81,19.5 mGy. The estimated cumulative dose (DLP [mGy-cm]) is 0.54,0.16,0.16,2.40,505. Estimated cumulative dose or total dose-length-product (DLP) is: 508 mGy-cm.

Intravenous Contrast Medium: Isovue 370. The amount administered to the patient is documented in the electronic medical record.

Cardiac Gating: Coronary Angiogram was performed using ECG-based tube current modulation with retrospective cardiac gating.

Image Reconstruction: Axial images of the Coronary Angiogram acquisition were reconstructed at thin (approximately 0.5-1 mm) slice thicknesses at the targeted R-R interval. Lower resolution axial images were also reconstructed of the entire R-R interval (0-90%).

3D reformations consisting of curved and multiplanar reformations, maximum intensity projections, and volume rendered images were performed on an independent workstation and corroborate the findings made in the report of the source images.

FINDINGS:

Prior Surgery/Intervention: None.

CORONARY ARTERIES:

Left Main: Arises from its expected left coronary sinus. Slightly high origin near the sinotubular junction. Patent without significant narrowing.

Left Anterior Descending: Patent without significant narrowing. Superficial partial bridging of the mid LAD. No complete myocardial bridge.

Left Circumflex: Patent without significant narrowing.

Right: Anomalous origin from the left coronary cusp, separate from the left main origin, slightly high near the sinotubular junction, with an interarterial course for approximately ~1 cm. Slightly acute takeoff angle but no significant narrowing of the interarterial segment. Otherwise, RCA is patent without significant narrowing. Right dominant coronary arterial system.

EXTRACORONARY CARDIOVASCULAR

Heart: Normal in size. Normal in architecture.

Pericardium: No pericardial effusion.

Visualized Aorta: No significant abnormality.

Pulmonary Arteries: Not enlarged. No obvious central filling defects on this non-dedicated study to suggest pulmonary embolism.

OTHER

Medical Devices: None.

Mediastinal/Hilar Lymph Nodes: No pathologically enlarged lymph nodes.

Other Mediastinal Structures: No significant abnormality.

Lung Parenchyma: Minimal subsegmental atelectasis at the left lung base.

Airways: Central airways are patent. Mild bronchial wall thickening.

Pleura: No pleural effusions.

Chest Wall: Mild prominence of bilateral breast tissue.

Visualized Abdomen: No significant abnormality on this limited, non-dedicated study.

Bones: No aggressive osseous lesions.

IMPRESSION:

* Anomalous right coronary artery origin from the left coronary cusp, separate from the left main takeoff, with an interarterial course. RCA origin is slightly high near the sinotubular junction. Takeoff angle is slightly acute, but no significant narrowing of the interarterial segment. Right coronary artery dominance.

* Patent left coronary system without stenosis. Superficial partial bridging of the mid LAD, but no complete (grade III) myocardial bridge.

* Structurally normal heart.

I, the attending signed below, have personally reviewed the images and agree with the report transcribed above.

Interpreted by Attending Radiologist: Evan J Zucker, MD

Electronically Signed: 8/8/2025 4:25 PM

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Approval Date : 8/8/2025 4:25 PM